NCCP 00794 · VERSION 3

Sacituzumab govitecan (Trodelvy)

Two treatment visits in each three-week cycle: Day 1, then Day 8. There is no planned infusion on Day 15. The next cycle starts after Day 21 if the team confirms it is appropriate.

BASELINE → TREATMENT → FOLLOW-UP

When checks happen

Reference timing, not booked appointments. Ask who requests each test, when it is due, who reviews the result and whom to contact if it is missing. Delays can change cycle numbers; time-based monitoring remains time-based.

Check	Before starting	First cycle / month	Early treatment	Ongoing treatment	Change / completion
Full blood count / neutrophils	Baseline blood count	Check before Day 1 and assess ANC before Day 8; the thresholds differ	Repeat the Day 1 / Day 8 assessment each cycle; extra counts if low or unwell	Continue pre-cycle count and Day 8 ANC assessment	Repeat until recovery if treatment is held; agree restart with the team
Kidney and liver tests	Baseline renal and liver profile	Before the cycle; repeat sooner for dehydration or symptoms	Before every cycle	Before every cycle; clinician-directed additional tests	Reassess any abnormality before changing or restarting treatment
Glucose / electrolytes	Glucose, electrolytes, magnesium, calcium, phosphate	Baseline results reviewed; repeat if diarrhoea or dehydration	Every second cycle under NCCP; sooner if clinically needed	Every second cycle, with symptom-driven additions	Recheck and replace losses after significant diarrhoea/vomiting
Infusion observation	Review allergy / reaction history and premedicine plan	First infusion about 3 hours, then ≥30-minute observation	Later infusions 1–2 hours if tolerated; ≥30-minute observation each time	Continue observation at every infusion	Any significant reaction changes the future infusion plan
ECG / echo	ECG if clinically indicated; no routine drug-specific echo	Repeat only if clinically indicated	Add testing for symptoms, risk factors or other medicines	No fixed echo schedule for sacituzumab alone	Investigate new cardiac symptoms appropriately
Pregnancy / fertility	Pregnancy test where relevant; fertility discussion and contraception	Repeat pregnancy test before the cycle where relevant	Pregnancy test before each cycle where relevant	Continue contraception and pre-cycle testing where relevant	Contraception continues for the specified period after the final dose
Response imaging / markers	Record measurable sites and baseline imaging	Agree first response review	Usually every 2–4 months; individualise	Adapt to disease pace; earlier for symptoms	Markers supplement, never replace, assessment

THE WHOLE TREATMENT JOURNEY

Sacituzumab govitecan (Trodelvy): the whole pathway

Previously treated unresectable or metastatic triple-negative breast cancer

Two treatment visits in each three-week cycle: Day 1, then Day 8. There is no planned infusion on Day 15. The next cycle starts after Day 21 if the team confirms it is appropriate.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	First cycle / month	Early treatment	Ongoing treatment	Change / completion
Treatment NCCP	Confirm exact indication and biomarkers	10 mg/kg intravenously on days 1 and 8 of a 21-day cycle; prescribed reductions may apply	Two treatment visits in each three-week cycle: Day 1, then Day 8. There is no planned infusion on Day 15. The next cycle starts after Day 21 if the team confirms it is appropriate.	Continue with safety / benefit review	Treatment continues while it is helping and side effects remain acceptable. There is no fixed number of cycles; the oncology team reviews benefit, scans and tolerability.
Surgery / radiotherapy ESMO_MBC2023	Only for an agreed local-control or symptom need	Teams decide sequence / any treatment interruption	No routine surgery or RT implied	Review any local-treatment effects	Reassess if goals or symptoms change
Safety checks NCCP · SMPC	Baseline bloods and individual risks	Early checks from the monitoring table	Regimen-specific checks continue	Review results before continuation	Resolve abnormalities / agree next plan
Scans / follow-up ESMO_MBC2023	Confirm baseline and treatment setting	Agree the next imaging milestone	Use the matching surveillance / response plan	Review benefit and symptoms	Continue setting-specific follow-up

What each check is for

1 **Blood tests**

Before the first treatment: full blood count, kidney and liver blood tests, glucose and electrolytes including magnesium, calcium and phosphate. Pregnancy testing is needed where relevant. ECG is added if clinically indicated. The team checks infection, medicines, previous low blood counts and whether preventive G-CSF injections are appropriate. Blood count, kidney and liver tests are checked before each cycle; the neutrophil count must also meet the separate Day 8 threshold before that infusion. Glucose and the listed electrolytes are checked every second cycle. Pregnancy testing is repeated before each cycle where relevant. Review diarrhoea, hydration, nausea, oral intake, fever and infection before each infusion. Scans follow the individual response-assessment plan.

Sources: NCCP · SMPC

2 **Heart checks**

An ECG is used when clinically indicated. Sacituzumab alone does not require a routine echocardiogram schedule. Cardiac symptoms or other medicines may change the plan.

Sources: NCCP · SMPC

3 **Imaging**

Response assessment in advanced breast cancer is generally every two to four months, adapted to disease dynamics, sites, symptoms and treatment. Less frequent monitoring may suit indolent disease. New symptoms or suspected progression need earlier assessment. The team chooses the imaging method; this is not an automatic three-month CT chest/abdomen/pelvis order.

Sources: ESMO_MBC2023

4 **Bone health**

A routine DXA scan is not a sacituzumab-specific requirement. Bone health, bone metastases and any bone-strengthening treatment need their own individual plan.

Sources: NCCP · SMPC

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.